

Nivolumab + ipilimumab + pemetrexed + carboplatin

Lung · NCCP 00713 · version 4

Your medicines

Your treatment schedule

A picture of side effects

Numbers connect each symptom box to the relevant anatomical area. Tell your team about new or worsening symptoms.

Nivolumab / ipilimumab · immune effects

Pemetrexed / carboplatin · chemotherapy

1 Hormones / brain

Nivolumab / ipilimumab

Fatigue, dizziness, headache, thirst or passing more urine can indicate immune endocrine toxicity.

2 Lungs

Nivolumab / ipilimumab

New cough or breathlessness needs prompt assessment for pneumonitis and other causes.

3 Heart / muscles

Nivolumab / ipilimumab

Chest pain, palpitations, fainting or marked muscle weakness needs urgent assessment.

4 Liver / bowel

Nivolumab / ipilimumab

Jaundice, dark urine, persistent diarrhoea or abdominal pain can indicate immune inflammation.

5 Kidneys

Pemetrexed / immune treatment

Kidney function is monitored. Report reduced urine, swelling or significant dehydration.

6 Blood / infection

Pemetrexed / carboplatin

Fever, shivers, bruising or sudden illness can reflect low blood counts.

7 Mouth / skin

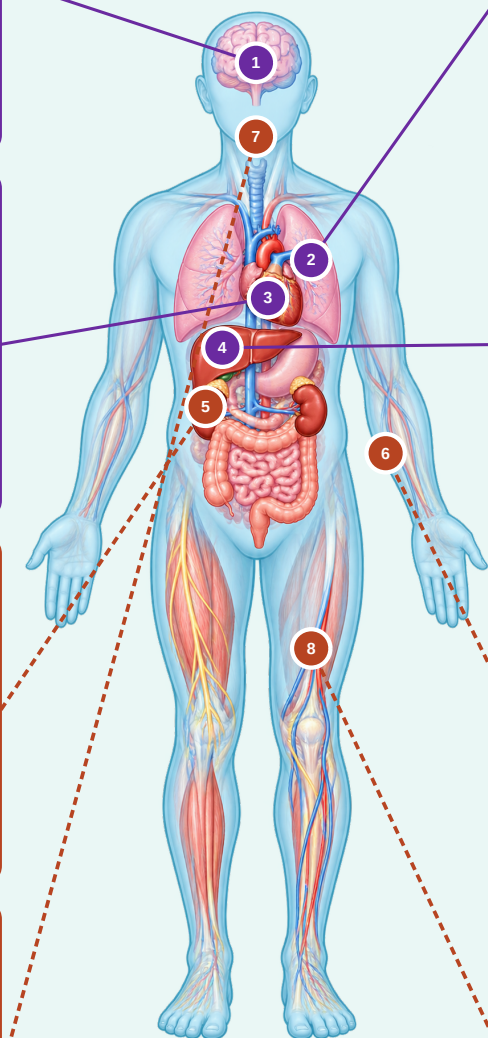
Pemetrexed / immune treatment

Mouth soreness or significant rash should be reported; severe blistering skin needs urgent review.

8 Infusion / allergy

Carboplatin / immunotherapy

Wheeze, swelling, dizziness or an acute infusion reaction needs immediate assessment.



Selected effects only: the same symptom can have more than one cause, and not every possible side effect is shown.

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TREATMENT JOURNEY

Cycle 1 · day 1	Cycle 1 · day 22	After cycle 1	Duration
Nivolumab + ipilimumab + pemetrexed + carboplatin.	Nivolumab + pemetrexed + carboplatin.	Continue nivolumab on days 1 and 22 and ipilimumab on day 1 of each 42-day cycle.	Immunotherapy continues until progression/unacceptable toxicity or up to 24 months in patients without progression, according to the NCCP

IMPORTANT MEDICINE-SPECIFIC EFFECTS

Nivolumab + ipilimumab - immune effects

- Immune inflammation: lungs, bowel, liver, hormone glands, kidneys, skin, heart, nerves or muscles.
- New immune symptoms can begin during treatment or after treatment has stopped.

Pemetrexed

- Low blood counts can increase infection, anaemia or bruising risk.
- Mouth/gut inflammation and skin rash can occur.
- Kidney function can deteriorate and is monitored.

Carboplatin

- Low blood counts, especially neutrophils and platelets, can occur.
- Allergic infusion reactions can occur, particularly after repeated exposure.
- Nausea and vomiting may occur despite preventative medicines.

OTHER / SHARED TREATMENT EFFECTS

The anatomy page highlights important symptom patterns for this exact regimen, but it cannot show every possible effect. Fatigue, nausea, appetite change, mouth/bowel symptoms or blood-count changes may occur depending on the medicines used. Report new or worsening symptoms even if they are not listed.

Monitoring / medicines

- Folic acid and vitamin B12 support are required with pemetrexed.
- Blood count, kidney/liver profile and endocrine monitoring support treatment safety.
- Use the regimen-specific antiemetic prescription for the chemotherapy phase.

Bring / track

- Immune adverse events can begin during treatment or after it stops; report new symptoms early.

WHEN TO CALL - DO NOT WAIT FOR THE NEXT APPOINTMENT

Contact oncology urgently for fever, new breathlessness, persistent diarrhoea, jaundice, severe rash, chest pain, marked weakness, confusion, reduced urine or rapidly worsening symptoms. For severe breathing difficulty, collapse, seizure or severe chest pain call 112/999.

Consent discussion support

Use the approved consent process to document treatment aim, expected benefit, material risks, alternatives, questions and the patient's decision.

Scan to open the regimen-specific patient portal

<https://sactcheck.com/patient/00713/>

Official NCCP 00713 v4

SACTCheck v0.81.2 · patient information prototype for clinical review. Follow your oncology team and local emergency instructions.

